INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Letrozole · daily hormone therapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Regimen source
NCCP 00371
Version 4

Continuous daily tablets

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

After surgery, letrozole can reduce the risk of hormone-sensitive breast cancer returning. Before surgery, it can be used in a selected treatment plan to shrink the cancer and prepare for an operation. In advanced disease, the aim is cancer and symptom control. Each setting has its own review and duration plan.

HORMONE THERAPY · AROMATASE INHIBITOR

Letrozole

Reduces the oestrogen available to stimulate cancer cells

Aromatase normally makes oestrogen → The medicine blocks aromatase → Less oestrogen reaches cancer-cell receptors

Letrozole blocks aromatase, reducing the oestrogen made in tissues after menopause and the stimulation of hormone-sensitive breast cancer cells. This can be useful in different settings, but a before-surgery course needs response and surgical reviews, while treatment after surgery has a planned total duration. It does not reliably switch off functioning ovaries; ovarian suppression needs its own treatment plan.

Tell the team if joint pain, hot flushes, vaginal dryness or sleep disturbance affects daily life. Bone thinning may cause no early symptoms, so keep the agreed bone-health reviews.

The treatment being discussed

Who this information covers

Letrozole is used for selected postmenopausal hormone-sensitive breast cancer before surgery, after surgery, as extended treatment or for advanced disease. Before-surgery treatment also depends on HER2 results, whether chemotherapy is suitable and whether surgery should happen immediately; the team checks these details.

NCCP 00371 v4 · official regimen

Medicine and schedule

Take letrozole once daily continuously. The prescriber confirms whether the aim is before surgery, after surgery, extended adjuvant treatment or advanced-disease control. Take with or without food. Take a forgotten dose when remembered unless the next dose is within about two to three hours; then skip it. Do not take two doses together.

NCCP 00371 v4 · official regimen

Review and duration

Take with or without food. Take a forgotten dose when remembered unless the next dose is within about two to three hours; then skip it. Do not take two doses together. A before-surgery response review, an adjuvant duration review and an advanced-disease scan answer different questions; use the milestones for your setting.

NCCP 00371 v4 · official regimen

Individual risks and support

The usual prescription is once daily, but severe liver impairment can require a different frequency. Use the frequency on your prescription and confirm any change with the prescriber or pharmacist. An already reduced or unusual schedule must be reviewed before it is changed. Confirmed menopause is needed for this monotherapy; any ovarian suppression has its own prescription and monitoring.

NCCP 00371 v4 · official regimen

Why this endocrine treatment is offered

For hormone-sensitive early breast cancer, endocrine treatment helps reduce the chance of the cancer returning. For advanced disease, it can help control cancer and symptoms.

Your likely benefit depends on your cancer, previous treatment and health. The treating team explains why this option fits your plan.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Report new tendon pain or swelling promptly and protect the affected area until assessed; sudden loss of tendon function needs urgent assessment. Also report limiting joint pain, a possible fracture, jaundice or new vaginal bleeding. New breast changes during before-surgery treatment need review rather than waiting for the planned end date. Severe allergic symptoms, chest pain or major breathing difficulty require urgent help.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Read the row for your treatment setting. Columns show order, not equal lengths of time. Agree the due window, booking team, result reviewer and whom to contact if a check is missing.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00371 v4 · official regimen · HPRA Letrozole SmPC cited by NCCP · Pan-Asian adapted ESMO early breast cancer guidelines · Pan-Asian adapted ESMO metastatic breast cancer guidelines · NICE NG 101: early and locally advanced breast cancer · ESO-ESMO ABC 5 consensus: tumour-marker interpretation · IOF/CABS/ECTS/IEG/ESCEO/IMS/SIOG updated joint statement on aromatase-inhibitor-associated bone loss

THE WHOLE TREATMENT JOURNEY

Letrozole · daily hormone therapy: treatment and checks

Your treatment pattern

Continuous daily tablets

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Postmenopausal hormone-sensitive breast cancer; setting-specific pathways

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Treatment plan NCCP 00371	Confirm indication and prescription	Start the agreed daily / injection schedule	Continue unless prescriber changes the plan	Review duration, benefit and tolerability	Agree continuation, switch or completion
Bloods / symptom review NCCP 00371	full blood count (FBC), renal and liver profile	Review tolerability and medicines	Further tests when clinically indicated	Review symptoms, adherence and results	Follow unresolved abnormalities
electrocardiogram (ECG) / echo NCCP 00371	No routine ECG / echo for this medicine alone	Investigate new cardiac symptoms	Further cardiac tests by risk / symptoms	Companion-drug cardiac rules remain separate	Follow abnormalities when indicated
Bone density / dual-energy X-ray absorptiometry (bone density) (DXA) NCCP 00371 · AIBL2025	Adjuvant aromatase inhibitor (AI): baseline DXA unless already on adjuvant bisphosphonates	Review previous DXA and fracture risk	No cycle-based DXA appointments	Lower-risk AI: reassess in 1–2 years; individualise	Continue bone care according to risk

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Early-breast surveillance NCCP 00371	Confirm early / curative setting	Set first follow-up mammogram date	Assess new symptoms promptly	Annual mammography of remaining breast tissue	No routine computed tomography (CT) / markers if asymptomatic
Advanced-disease response NCCP 00371	Document disease sites / baseline assessment	New symptoms can bring assessment forward	Generally every 2–4 months; individualise	Adapt interval and imaging to disease / benefit	Markers supplement, never replace, assessment
Before-surgery pathway only NCCP 00371	Confirm hormone receptors, HER2 and the chemotherapy / surgery plan	Agree clinical response / surgery reviews	Assess response during 4–8 months	Inadequate response: surgery / alternative review	Proceed to agreed operation / next treatment
Early: radiotherapy ESMO_EBC2024	Review surgery / pathology	Plan radiotherapy if indicated	Coordinate endocrine timing	Follow the early-breast plan	Review late effects when relevant

What each check is for

1 Confirm the plan

Before starting or changing treatment

Letrozole is used for selected postmenopausal hormone-sensitive breast cancer before surgery, after surgery, as extended treatment or for advanced disease. Before-surgery treatment also depends on HER2 results, whether chemotherapy is suitable and whether surgery should happen immediately; the team checks these details.

Ask / act: Confirm treatment setting, medicines, menopause and the planned review.

2 Checks and symptoms

At the agreed review points

FBC, renal and liver profile; confirm menopause with hormone tests when uncertain. Assess fracture risk and baseline bone density, particularly for adjuvant AI treatment. The NICE adjuvant baseline-DXA recommendation has an exception for patients already receiving adjuvant bisphosphonates. FBC, renal / liver profile and lipids when clinically indicated. Review bone density during treatment and after treatment according to bone risk. Neoadjuvant response and surgical planning need explicit review milestones.

Ask / act: Agree who books tests, reviews results and responds to new symptoms.

3

Review ongoing benefit

At setting-specific decision points

After surgery, letrozole can reduce the risk of hormone-sensitive breast cancer returning. Before surgery, it can be used in a selected treatment plan to shrink the cancer and prepare for an operation. In advanced disease, the aim is cancer and symptom control. Each setting has its own review and duration plan.

Ask / **act:** Review duration, alternatives and patient priorities; do not stop or extend treatment automatically.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks throughout treatment

Your team arranges tests and explains the results. A planned check is not confirmation that it has been completed.

Treatment-specific checks

FBC, renal and liver profile; confirm menopause with hormone tests when uncertain. Assess fracture risk and baseline bone density, particularly for adjuvant AI treatment. The NICE adjuvant baseline-DXA recommendation has an exception for patients already receiving adjuvant bisphosphonates. FBC, renal / liver profile and lipids when clinically indicated. Review bone density during treatment and after treatment according to bone risk. Neoadjuvant response and surgical planning need explicit review milestones.

NCCP 00371 v4 · official regimen

Heart checks

ECG or other heart tests depend on symptoms and personal risk; this medicine alone does not require a routine HER2-style echocardiogram schedule.

NCCP 00371 v4 · official regimen

Bone health

Offer baseline DXA when starting an adjuvant aromatase inhibitor if not already receiving adjuvant bisphosphonates; assess fracture risk and relevant previous results. Under the 2025 joint guidance, patients with a T-score above –2.0 and no additional risk factors generally have risk and bone density reassessed every one to two years. Patients on bone-protective treatment generally have BMD reassessed about every two years. Fracture, substantial bone loss or a change in risk can require earlier review. The clinician selects the interval and any bone treatment; these are not automatic yearly scans for everyone.

NICE NG 101: early and locally advanced breast cancer · IOF/CABS/ECTS/IEG/ESCEO/IMS/SIOG updated joint statement on aromatase-inhibitor-associated bone loss

Your setting changes the pathway

Choose the pathway that your team has confirmed; these are not all extra treatments you must have.

Early-breast follow-up

After curative treatment, annual mammography is generally used for remaining breast tissue: both breasts after conservation or the opposite breast after unilateral mastectomy. The mastectomy side is not routinely mammographed. Routine CT, bone scans and tumour markers are not recommended for asymptomatic early-breast surveillance. New symptoms need assessment between appointments.

Pan-Asian adapted ESMO early breast cancer guidelines · NICE NG 101: early and locally advanced breast cancer

Surgery, chemotherapy and radiotherapy

In an early-breast pathway, chemotherapy, if indicated, precedes endocrine treatment. Surgery and its pathology guide adjuvant choices. Postoperative radiotherapy is usually delivered after recovery and any adjuvant chemotherapy; aromatase inhibitors may overlap radiotherapy under the agreed plan. Tamoxifen timing around radiotherapy is agreed by the treating teams. None of these additional treatments is automatically required by an endocrine prescription.

Pan-Asian adapted ESMO early breast cancer guidelines · eviQ 3650 v 6: adjuvant whole-breast radiotherapy

Advanced-disease scans

For advanced breast cancer, response assessment is generally every two to four months, adapted to disease pace, sites, symptoms and treatment. Indolent disease may be reviewed less often; new symptoms can prompt earlier scans. The team chooses the imaging method; this is not a universal three-month CT chest/abdomen/pelvis order.

Tumour markers

Tumour markers may supplement imaging and clinical assessment in advanced disease if they are informative, especially if elevated at baseline. They do not replace scans, and a marker change alone should not determine treatment failure. There is no universal fixed marker timetable.

Menopause, pregnancy and contraception

AIs alone require a confirmed postmenopausal endocrine state. Ovarian suppression is a separate treatment plan and must not be assumed from this card.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your endocrine treatment and review milestones

These stages show the order of care, not equal lengths of time. No dates or personal details are needed. Follow the prescribed tablet schedule between reviews; the treatment setting determines when it is reviewed or completed.

Treatment or check	Timing in this view
Prescribed tablets	**First weeks → During treatment → Longer-term review → Change / completion** Usual schedule: 2.5 mg once daily; follow a different frequency only if prescribed
Before the first dose	**Before starting** Confirm menopause, treatment setting, baseline bloods, medicines and bone-health plan
Symptoms and tolerability	**First weeks → During treatment → Longer-term review → Change / completion** May persist or change; report troublesome symptoms early
Planned reviews	**During treatment → Longer-term review → Change / completion** Review symptoms, adherence, bone health and the planned duration or disease response
Before surgery only	**First weeks → During treatment → Longer-term review → Change / completion** Assess response during the agreed course; inadequate response prompts surgery or alternative-treatment review

Take with or without food. Take a forgotten dose when remembered unless the next dose is within about two to three hours; then skip it. Do not take two doses together.

A before-surgery response review, an adjuvant duration review and an advanced-disease scan answer different questions; use the milestones for your setting.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Practical support

Symptoms and daily life

Support medicines

No routine antiemetic, steroid premedication or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) is required by this endocrine monotherapy. Ask for individual symptom support.

Tests and appointments

Ask who books each check, when it is due and who reviews it. Tell the team if an expected appointment has not arrived.

Taking tablets / attending injections

Practical treatment details

Support during treatment

Discuss activity and joint symptoms.

Questions to ask

Do I have a treatment-free week?

Letrozole is usually taken every day without a planned treatment-free week. Some people have a different prescribed frequency because of liver problems. Follow your own prescription and check before changing it.

When are bloods, ECGs or echoes needed?

FBC, renal and liver profile; confirm menopause with hormone tests when uncertain. Assess fracture risk and baseline bone density, particularly for adjuvant AI treatment. The NICE adjuvant baseline-DXA recommendation has an exception for patients already receiving adjuvant bisphosphonates. FBC, renal / liver profile and lipids when clinically indicated. Review bone density during treatment and after treatment according to bone risk. Neoadjuvant response and surgical planning need explicit review milestones. There is no routine ECG or echo schedule for letrozole alone. Assess cardiovascular risk factors and new symptoms as clinically indicated.

Which scans apply to me?

After curative treatment, annual mammography is generally used for remaining breast tissue: both breasts after conservation or the opposite breast after unilateral mastectomy. The mastectomy side is not routinely mammographed. Routine CT, bone scans and tumour markers are not recommended for asymptomatic early-breast surveillance. New symptoms need assessment between appointments. For advanced breast cancer, response assessment is generally every two to four months, adapted to disease pace, sites, symptoms and treatment. Indolent disease may be reviewed less often; new symptoms can prompt earlier scans. The team chooses the imaging method; this is not a universal three-month CT chest/abdomen/pelvis order.

How long will treatment continue?

For the NCCP neoadjuvant pathway, reassess response during a four-to-eight-month course and discuss surgery or alternatives if response is inadequate. Adjuvant / extended duration and advanced-disease continuation follow separate plans.

Do I need a port or home pump?

No port or home infusion pump is needed for this medicine itself. Any separately prescribed medicine has its own access requirements.

Can I use HRT, vaginal oestrogen or supplements?

Check with the oncology team before starting any hormone-containing treatment, including vaginal oestrogen, or any new medicine or supplement. Some can oppose this treatment or change how it works. The team can suggest symptom treatments and decide whether specialist review is needed.

What if periods return or menopause is uncertain?

Tell the team promptly about a return of periods or new vaginal bleeding. No periods after chemotherapy does not always mean the ovaries have permanently stopped working. Hormone blood tests may be needed. Ovarian suppression, if prescribed, has its own schedule and monitoring.

What should I do if symptoms make tablets difficult to continue?

Tell the team early about joint pain, hot flushes, sleep, vaginal or sexual symptoms, fatigue or missed tablets. Support, a supervised break or a change of medicine may be possible. Agree the plan with the prescriber rather than changing or stopping treatment yourself.

Will before-surgery treatment delay my operation automatically?

No. Your team agrees when to assess the response and when surgery should take place. The NCCP pathway describes a course of about four to eight months to allow tumour reduction; inadequate response requires a discussion of surgery or another treatment. New symptoms or changes should be reported sooner.

Do I need calcium and vitamin D?

The NCCP regimen advises daily calcium and vitamin D supplementation. Ask the team or pharmacist to confirm the suitable product and dose, taking account of your diet, blood results, kidney health and any supplements you already take. Bone-protective medicines and bone-density checks follow your individual fracture-risk plan.

Explain the plan back: What is your treatment schedule, which checks apply to your setting, and who books and reviews them?

Take letrozole once daily continuously. The prescriber confirms whether the aim is before surgery, after surgery, extended adjuvant treatment or advanced-disease control.

> Clinician supportive-care discussion draft. This is not an issued prescription.

Minimal emetogenic risk

No routine prophylactic antiemetic, corticosteroid premedication or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) order is populated for this endocrine monotherapy. Review persistent nausea or other symptoms and prescribe individually if needed.

Medicine-specific support

- Tell the team if joint pain, hot flushes, vaginal dryness or sleep disturbance affects daily life. Bone thinning may cause no early symptoms, so keep the agreed bone-health reviews.
- No routine antiemetic, steroid premedication or G-CSF is required by this endocrine monotherapy. Ask for individual symptom support.
- Ask who books each check, when it is due and who reviews it. Tell the team if an expected appointment has not arrived.
- The usual prescription is once daily, but severe liver impairment can require a different frequency. Use the frequency on your prescription and confirm any change with the prescriber or pharmacist. An already reduced or unusual schedule must be reviewed before it is changed. Confirmed menopause is needed for this monotherapy; any ovarian suppression has its own prescription and monitoring.
- NCCP no renal adjustment including haemodialysis expected; HPRA creatinine clearance (CrCl) <10 lacks sufficient data and requires careful benefit/risk review. Child–Pugh A/B no adjustment. C: NCCP 50% original average dose = 2.5 mg every other day; HPRA close supervision without this numerical instruction. Reconcile current reduced/custom schedule first.

Individual plan

Prescriber / date: _____ Pharmacy review: _____

Verify allergies, interactions, relevant organ function, treatment setting and the exact medicine before prescribing.

NCCP 00371 v4 · Monitoring and treatment map

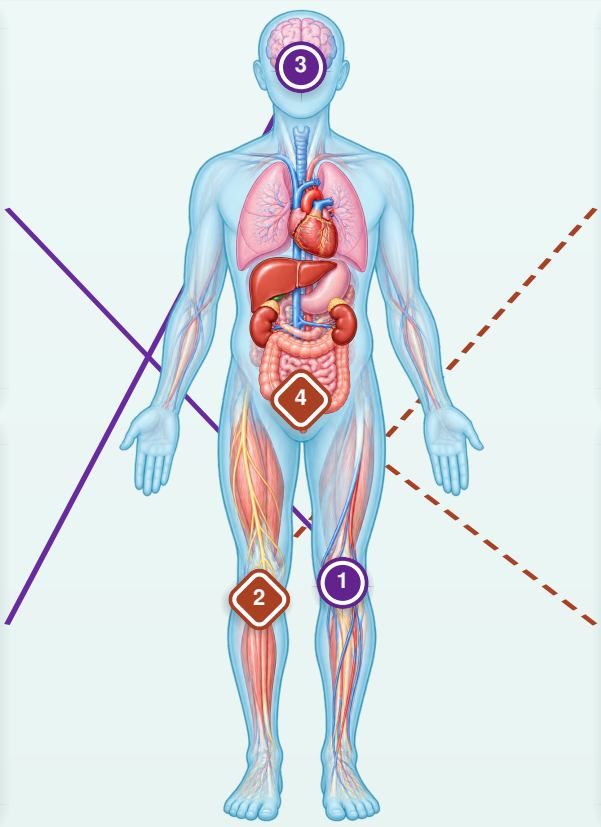
Letrozole · daily hormone therapy

Take letrozole once daily continuously. The prescriber confirms whether the aim is before surgery, after surgery, extended adjuvant treatment or advanced-disease control.

Hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Joints and muscles

Letrozole

Pain or stiffness can affect activity; report persistent or limiting symptoms.

2 Bone strength

Letrozole

Bone loss may be silent. Your risk assessment and bone-density plan matter.

3 Hot flushes / sleep

Letrozole

Warm flushes and disrupted sleep can be treated; tell the team.

4 Vaginal / sexual symptoms

Letrozole

Dryness or discomfort deserves support. Ask before using hormone-containing treatments.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report new tendon pain or swelling promptly and protect the affected area until assessed; sudden loss of tendon function needs urgent assessment. Also report limiting joint pain, a possible fracture, jaundice or new vaginal bleeding. New breast changes during before-surgery treatment need review rather than waiting for the planned end date. Severe allergic symptoms, chest pain or major breathing difficulty require urgent help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00371 v4 · official regimen**
   https://healthservice.hse.ie/documents/6817/371_Letrozole_.pdf
2. **HPRA Letrozole SmPC cited by NCCP**
   https://assets.hpra.ie/products/Human/21055/Licence_PA0749-085-001_12112019144136.pdf
3. **Pan-Asian adapted ESMO early breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC11145753/
4. **Pan-Asian adapted ESMO metastatic breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10186487/
5. **NICE NG 101: early and locally advanced breast cancer**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
6. **ESO-ESMO ABC 5 consensus: tumour-marker interpretation**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC7510449/
7. **IOF/CABS/ECTS/IEG/ESCEO/IMS/SIOG updated joint statement on aromatase-inhibitor-associated bone loss**
   https://www.esceo.org/sites/esceo/files/pdf/1-s2.0-S2212137425000351-main.pdf
8. **BIG 1-98: letrozole and tamoxifen alone and in sequence**
   https://pubmed.ncbi.nlm.nih.gov/22018631/
9. **MA.17: letrozole following five years of tamoxifen**
   https://pubmed.ncbi.nlm.nih.gov/16145047/
10. **P 024: preoperative letrozole versus tamoxifen**
    https://pubmed.ncbi.nlm.nih.gov/11822750/
11. **eviQ 3650 v 6: adjuvant whole-breast radiotherapy**
    https://www.eviq.org.au/radiation-oncology/breast/3650-breast-invasive-cancer-adjuvant-whole-breast
12. **BIG 1-98: long-term follow-up of letrozole versus tamoxifen**
    https://pubmed.ncbi.nlm.nih.gov/30475668/